



STRATEGY & OPERATIONS

Vytalix Partnership Strategy

Vytalix company-build documentation · reference

12_PARTNERSHIP_STRATEGY

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

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Owner: Chief Strategy Officer · **Status:** PROPOSED v1 · **Date:** 8 September 2026 · **Governing file:** [/FACTS_BASE.md](#) (KNOWN / VERIFIED / ASSUMPTION / ESTIMATE / PROPOSED / TO VALIDATE apply throughout). **Vytalix has no partners today** — no MoU, LOI, contract, grant, membership or introduction with any organisation named in this document (VERIFIED absence, FACTS_BASE §1). Every organisation below is named as a public institution and a target; nothing here may be read, quoted or presented as a relationship. Every score is a desk ESTIMATE to be re-scored after first contact.

Executive view

- 1. **Partnerships are the route to market for everything Vytalix cannot buy or build alone: NHS access, clinical credibility, an African cohort, evidence, and non-dilutive money.** A solo, pre-formation founder has no references; partners substitute for them. The plan therefore treats partnerships as the second commercial engine beside founder-led services, not as a corporate-development afterthought.
- 2. **Twenty-five organisations were scored on seven weighted criteria (§3).** The top tier for the next 12 months is a regional Health Innovation Network (86/100), Five X More (76), Africa's Talking (76), Society for Family Health Nigeria (74) and the DigitalHealth.London Accelerator (73) — organisations that are reachable by a solo founder, low-cost to engage and directly unlock either NHS introductions, community co-design, a Nigerian cohort or the messaging rail MaternaLink (in development) needs.
- 3. **Three relationships are gated, not scored down: mDoc, MSD for Mothers and any government.** mDoc is the funded incumbent in Ekiti (VERIFIED); MSD for Mothers funds it. Both are approached only with complementarity language, never with the March 2026 Ekiti proposal, and only after the MaternaLink IP question is settled (FACTS_BASE §1). Government relationships are pursued through implementing partners and donors, and an MoU is never described as a "government partnership".
- 4. **Six partnership models are used (§4) and each has a default contract form and an economic rule:** referral (fee-transparent, Bribery Act-safe), co-development (paid community partners, joint IP terms), distribution (revenue share 10–30%, ESTIMATE), data (controller/processor clarity, no individual data to insurers), research (co-applicant, shared authorship, ethics first), funding (grants where a UK entity and a clinical partner exist). Charities and community organisations are always paid for co-design; unpaid "endorsement" is never asked for.
- 5. **The 12-month plan (§5) targets, by month 12: one Health Innovation Network programme place, two paid community co-design agreements, one university co-applicant agreement, one Nigerian implementing-partner LOI for a cohort, one messaging-gateway pilot account, one NHS design-partner conversation opened through a clinical adviser, and two joint grant bids submitted (PROPOSED).** None of these is revenue in year 1; all of them are what an investor, a trust or a ministry will ask to see in year 2.

0. Method, definitions and rules of engagement

Term	Meaning in this document
Partner	An organisation with which Vytalix has a written arrangement (LOI, MoU, agreement, programme place, grant award, sub-contract). Until a document exists, an organisation is a target , and a conversation is a conversation .
Ecosystem ring	Ring 1 = controls access to buyers or users; Ring 2 = confers credibility or evidence; Ring 3 = supplies money, rails or reach. Many organisations sit in two rings.
Model	One of six structures in §4: referral, co-development, distribution, data, research, funding (plus sub-contracting and content licensing as variants).
Score	Seven weighted criteria in §2; 0–100; ESTIMATE.
Horizon	Now (first contact within 90 days of incorporation); 6m ; 12m ; 24m+ .

Rules of engagement (apply to every outreach, deck, website page and investor conversation):

- 1. Never name an organisation as a partner, supporter, client, endorser or "in discussion" without a signed document or their written permission for the wording (FACTS_BASE §4).
- 2. Never restate the March 2026 Ekiti proposal (£4.5m / £300 per woman per year) to any organisation; it is UNVALIDATED and it is not Vytalix's document (FACTS_BASE §1).
- 3. Never approach a donor, ministry or NGO about MaternaLink (in development) until the IP relationship with David Agunede is documented (FACTS_BASE §1). Services and E-Learning partnerships are not gated.
- 4. Pay community organisations and charities for co-design, content and listening exercises; do not ask for endorsement, logos or unpaid advocacy.
- 5. Referral fees, commissions and revenue shares are written down, disclosed to the end customer where NHS conflict-of-interest guidance or the Bribery Act 2010 requires it, and never paid to public officials or their relatives. A partnership register (see §5.6) records every arrangement.
- 6. Contact details are taken only from public sources; where none exist, the CRM records NOT PUBLICLY AVAILABLE.
- 7. Every conversation is logged in the CRM within 24 hours with the next dated action ([/docs/13_SALES_ENGINE.md](#) discipline applies to partners as to customers).

1. Ecosystem map

Organisations are named as public institutions. "Role for Vytalix" is a proposal. The final column states which Vytalix pillar the relationship serves: **Adv** (Advisory), **Con** (Consulting), **HS** (Health Solutions — MaternaLink, in development), **EL** (E-Learning).

1.1 United Kingdom — NHS system and public bodies

Partner class	Examples (institutions only)	What they control	Role for Vytalix (PROPOSED)	Ring	Pillar	Horizon
NHS trusts (maternity services)	~120 trusts in England provide maternity care (ESTIMATE; /docs/06_CUSTOMER_SEGMENTS.md §1.1)	Design-partner slots; Clinical Safety Officer sign-off (DCB0160); service-evaluation data; reference status	One unpaid design partner (one community midwifery team) for MaternaLink v1; consulting buyer for C3/C8	1, 2	HS, Con	12m (conversation), 24m+ (paid)
Integrated Care Boards / Local Maternity and Neonatal Systems (LMNS)	42 ICBs in England, each hosting an LMNS (/docs/05_MARKET_STRATEGY.md §1.1)	Equity action plans; improvement budgets; programme sponsorship	Sub-contracted or direct C8 Health Inequalities Review; sponsor for a multi-trust design partnership	1	Con, HS	12m
Maternity and Neonatal Voices Partnerships (MNVPs)	One per LMNS (structure TO VALIDATE locally)	The voice of service users in NHS maternity decisions	Listening-exercise partner for C8; co-design input for MaternaLink (paid for time)	2	Con, HS	6m
Health Innovation Networks (HINs)	15 in England, formerly AHSNs (count TO VALIDATE); e.g., Health Innovation Network South London, Health Innovation Manchester, Health Innovation Oxford and Thames Valley	Innovation programmes; trust introductions; real-world evaluation support; NHS market-access advice	Programme place → introductions → evaluation partner; also a quasi-competitor in advice (/docs/11_COMPETITIVE_INTELLIGENCE.md §4)	1, 2	HS, Con	Now
NIHR	Applied Research Collaborations (ARCs); Invention for Innovation (i4i); HealthTech Research Centres (HRCs); Research Design Service successor bodies (names TO VALIDATE)	Grant funding for NHS-partnered innovation; methodological support; evaluation credibility	Funding and evaluation route for the UK feasibility study (2027–28); requires an NHS partner and a UK entity	2, 3	HS	6m (prepare), 12m (bid)
NHS England / DHSC bodies	Three Year Delivery Plan owners; digital maternity record standard; national maternity investigation (2025–26)	Policy pull; standards (PRSB digital maternity record)	Alignment, not partnership: design to the standard; respond to consultations	2	HS	12m
NICE	Evidence Standards Framework (ESF) for digital health technologies	Evidence-tier expectations	Alignment target for the evidence plan; no partnership sought	2	HS	12m
Professional bodies	Royal College of Midwives (RCM); Royal College of Obstetricians and Gynaecologists (RCOG)	Professional trust; content standards; CPD channels; conference access	Content-licensing conversation (RCOG patient information TO VALIDATE); CPD/E-Learning alignment; speaking slots	2	EL, HS	6m
Accelerators and programmes	DigitalHealth.London Accelerator; NHS Clinical Entrepreneur Programme (eligibility TO VALIDATE); NHS Innovation Accelerator (later stage); SBRI Healthcare (funding competitions)	Cohort places; NHS mentors; investor and trust exposure	Apply once incorporated; use for introductions and credibility; SBRI as a funding route	1, 2, 3	HS, Adv	Now
Standards and interoperability	PRSB; HL7 UK / FHIR UK Core; NHS DTAC owners	Interoperability standards; assurance expectations	Design-to-standard; DTAC readiness for v1; integration (v2)	2	HS	12m

1.2 United Kingdom — private providers, charities, universities, insurers, employers

Partner class	Examples (institutions only)	What they control	Role for Vytalix (PROPOSED)	Ring	Pillar	Horizon
Private hospital groups and maternity units	HCA Healthcare UK (The Portland Hospital), Spire, Nuffield Health, Circle Health; the Kensington Wing and Lindo Wing (NHS private units)	Fast decisions; premium client experience budgets; fewer IG barriers	Consulting buyer (C1) and a faster MaternaLink pilot market (/docs/06_CUSTOMER_SEGMENTS.md §1.2–1.3); distribution partner for E-Learning to staff	1	Con, HS	6m–12m
Maternal-health research charities	Tommy's; Wellbeing of Women; Sands; Bliss	Research funding; public trust; content	Research co-funding conversations; content signposting and licensing (never duplicate free information); joint events	2, 3	HS, EL	6m
Advocacy and community organisations (Black maternal health, rights)	Five X More; Birthrights; Maternity Action; Black Mothers Matter-type community groups (TO VALIDATE current organisations); Best Beginnings (Baby Buddy app)	Community trust; lived-experience input; policy voice	Paid co-design, listening exercises and content review for MaternaLink and for C8 reviews; content signposting (Baby Buddy)	2	HS, Con	Now
Parenting and antenatal organisations	NCT	Antenatal education reach; parent trust	Content and distribution conversation for the woman-facing layer (later)	1, 2	HS, EL	12m

Partner class	Examples (institutions only)	What they control	Role for Vytalix (PROPOSED)	Ring	Pillar	Horizon
Universities — maternal-health research groups	NPEU (University of Oxford); King's College London; University of Manchester; LSHTM MARCH Centre; City St George's, University of London (midwifery — TO VALIDATE current name); Liverpool School of Tropical Medicine (LSTM) for Africa evaluation	Methodology; ethics routes; NIHR/UKRI/Wellcome co-applications; students	Service-evaluation and feasibility-study partner (2027–28); v3 research-track partner; E-Learning licences and guest lectures	2, 3	HS, EL	6m
Insurers and benefits platforms (UK)	Bupa; AXA Health; Vitality; Aviva	Employer maternity benefits; member communications	Discovery interviews only in 2026; benefit-bundle distribution 24m+ (/docs/04_MATERNALINK_PRODUCT_STRATEGY .md \$14)	1, 3	HS	24m+
Employers and benefits consultancies	Large employers with DEI/maternity-return programmes; benefits consultancies	Workshop and E-Learning budgets	E-Learning and workshop distribution; no product claims	1	EL, Con	6m
Consultancies (as primes)	PA Consulting; Carnall Farrar; Deloitte (/docs/11_COMPETITIVE_INTELLIGENCE .md \$4)	NHS frameworks; large programmes	Sub-contract route for NHS consulting until Vytalix holds a framework place	1	Con	6m
Technology vendors (UK maternity)	System C (BadgerNet Maternity); K2 Medical Systems; Magentus (Euroking)	The maternity record in most trusts; integration gateways	Integration-first posture (v2); "layer beside the record"; possible channel	1	HS	12m

1.3 Africa — governments, programmes, NGOs, rails

Partner class	Examples (institutions only)	What they control	Role for Vytalix (PROPOSED)	Ring	Pillar	Horizon
Nigeria — federal	Federal Ministry of Health and Social Welfare; National Primary Health Care Development Agency (NPHCDA); National Health Insurance Authority (NHIA); digital-health governance bodies (current names TO VALIDATE)	National digital-health strategy; PHC standards; insurance rules	Alignment and registration; no early partnership sought	2	HS	12m
Nigeria — state	Ekiti State Ministry of Health (one prior conversation by David Agunede, outcome unknown); Lagos State MoH (works with mDoc); candidate states Oyo, Kano, Ondo, Osun, Kwara (/docs/05_MARKET_STRATEGY .md \$2.1); State Primary Health Care Development Agencies (SPHCDA); state health insurance agencies	LGA access; CHEW workforce; data ownership; budget lines (usually donor-backed)	Pilot host via an implementing partner; MoU → pilot agreement → contract sequence (/docs/04_MATERNALINK_PRODUCT_STRATEGY .md \$12)	1	HS, Adv	12m–24m+
Kenya	Ministry of Health; county health departments (47); Social Health Authority (SHA) — maternity coverage under transition (TO VALIDATE)	County procurement; facility networks	Second African market (months 18–36); county or NGO programme	1	HS	24m+
Ghana	Ghana Health Service; National Health Insurance Authority	Centralised buyer	"Prepare, do not enter" until 2028	1	HS	24m+
Incumbent programmes (complement, not compete)	mDoc (Digital Mom Project, Ekiti and Lagos, funded by MSD for Mothers — VERIFIED); Jacaranda Health (PROMPTS, Kenya); Helium Health (facility records, Nigeria)	Enrolled women; provider networks; donor goodwill	Complementarity conversations (facility-side coordination, handover, CHEW workflow); integration-ready posture with Helium	1, 2	HS	Now (conversation), 12m (agreement)
Implementing NGOs — Nigeria	Society for Family Health Nigeria; Wellbeing Foundation Africa; Nigeria Health Watch (convenor/advocacy, not implementer); Pathfinder International Nigeria; Jhpiego Nigeria	Cohorts in LGAs; CHEW training; donor reporting; ethics routes (NHREC/state)	The fastest route to a cohort: programme-licence host; joint grant applicant; evaluation co-author (/docs/04_MATERNALINK_PRODUCT_STRATEGY .md \$13)	1, 2	HS, Con	6m
Implementing NGOs — Kenya and regional	Amref Health Africa; Living Goods; Jacaranda Health; Population Services Kenya (TO VALIDATE current structure)	CHW networks; county relationships	Kenya entry partner (2027–28)	1	HS	24m+
International NGOs and technical agencies	Jhpiego; PATH; MSI Reproductive Choices; Population Council; Save the Children	Large maternal-health programmes; digital-health technical leadership (PATH); country offices	Sub-contract, joint bids, programme licence; evaluation credibility	1, 2	HS, Con	6m–12m
Multilateral programmes	WHO (Digital Adaptation Kit for antenatal care; SMART guidelines); UNFPA (maternal health and innovation calls); UNICEF (Office of Innovation, country programmes); World Bank Global Financing Facility (GFF)	Standards; funding windows; ministry influence	Design to WHO ANC digital adaptation kit; UNFPA/UNICEF innovation calls (eligibility TO VALIDATE); GFF-financed state plans as budget context	2, 3	HS	6m (alignment), 12m (calls)
Bilateral development organisations	FCDO (UK-funded health programmes and research); GIZ; USAID (restructured into the US State Department in 2025 — funding landscape TO VALIDATE); Global Affairs Canada via Grand Challenges Canada	Programme funding through primes; research calls	Sub-contract to primes; FCDO-funded research consortia via UK universities	3	HS, Con	12m

Partner class	Examples (institutions only)	What they control	Role for Vytalix (PROPOSED)	Ring	Pillar	Horizon
Foundations and corporate programmes	MSD for Mothers (funds mDoc — VERIFIED); Gates Foundation / Grand Challenges; Children's Investment Fund Foundation (CIFF); Wellcome; ELMA Philanthropies; Aliko Dangote Foundation; TY Danjuma Foundation (Nigeria; fit TO VALIDATE)	Non-dilutive funding; programme partnerships	Grant applications with an NGO or university lead; MSD for Mothers approached only with complementarity to its existing grantee	3	HS	6m (prepare), 12m (bid)
Telecoms and messaging rails	MTN Nigeria / MTN Foundation; Airtel Africa; Safaricom (M-Pesa; M-TIBA health-payments platform with CarePay); Africa's Talking; Termii; Twilio; Vodafone Foundation	SMS/USSD/WhatsApp/voice rails; sender-ID and DND compliance; zero-rating; health propositions	Gateway account (Now); telco bundle/white-label channel (24m+, /docs/01_BUSINESS_MODEL.md stream #13)	3	HS	Now (gateway), 24m+ (telco)
Insurers and HMOs — Africa	Nigeria: AXA Mansard Health, Hygeia HMO, Reliance Health, Leadway Health; state health insurance agencies; Kenya: Jubilee Health, Britam, SHA	Enrolled women with a financial interest in ANC adherence and facility delivery	Engagement-fee pricing with aggregate adherence dashboards; no individual clinical data; 24m+	1, 3	HS	24m+
Banks and development finance	Access Bank (women's banking programmes), Standard Chartered (community programmes), First Bank — CSR fit TO VALIDATE; British International Investment (BII); IFC	CSR budgets; later-stage capital	Opportunistic CSR co-funding of a cohort; DFI conversations only at Series A	3	HS	24m+
Diaspora and convening bodies	Nigerian and Kenyan clinician associations in the UK (e.g., MANSAG-type associations — TO VALIDATE); Nigeria Health Watch (Future of Health conference)	Trusted introductions; convening platforms	Advisory-board sourcing; speaking; warm introductions to state and NGO leaders	1, 2	HS, Adv	Now

1.4 Reading the map

- Only four classes are reachable and useful in the next 90 days without an entity or IP resolution:** Health Innovation Networks and accelerators (applications), community organisations (paid co-design for services work), messaging gateways (a developer account) and diaspora/convening bodies (introductions).
- Every African government relationship runs through a Ring 1 implementing partner or a Ring 3 funder;** Vytalix does not approach a ministry alone (/docs/04_MATERNALINK_PRODUCT_STRATEGY.md §12.1).
- Two incumbents (mDoc, Jacaranda) are partner targets first and competitors second;** the test is one documented conversation each before any further product spend in their markets (/docs/11_COMPETITIVE_INTELLIGENCE.md Priorities).

2. Partnership scoring framework (PROPOSED)

Seven criteria; weights sum to 100; each criterion scored 1–5 (5 = most favourable); **score = $\Sigma(\text{weight} \times \text{criterion score}) \div 5$** , giving 0–100. Cost and risk are scored inversely (5 = low cost, low risk).

#	Criterion	Weight	What "5" means	What "1" means
1	Strategic value	20	Unlocks a gate in the plan (NHS design partner, African cohort, evidence, IP-safe content) that nothing else unlocks	Nice to have; no gate opened
2	Revenue potential (24 months)	15	Direct fees, sub-contract days or programme licence $\geq$ £50k (ESTIMATE)	£0 and no plausible path
3	Credibility conferred	15	Named association would change a trust's, donor's or investor's view immediately	Unknown to buyers and investors
4	Access (can the founder reach a decision-maker within 30 days?)	15	Public application route or founder-level contact; answers in weeks	Requires an introduction Vytalix does not have; institutional gatekeeping
5	Time-to-value	15	Signed document or programme place within 3 months	12+ months to any document
6	Cost to engage (inverse)	10	Under 3 founder days and $<$ £1k to reach a decision	$>$ 20 founder days, match funding, travel or legal cost $>$ £10k
7	Risk (inverse)	10	No IP, reputational, dependency or conflict risk	Dependency on a competitor's goodwill, IP exposure, political or payment risk

Gates applied before scoring is used (a "no" on any gate pauses outreach regardless of score):

Gate	Applies to	Condition to pass
G1 — Entity	Any signed document; any grant	Vytalix incorporated, bank account, professional indemnity in place
G2 — IP	Any conversation about MaternaLink (in development)	Written agreement with David Agunede on MaternaLink IP (FACTS_BASE §1)
G3 — Claims	Any external material	Intended-use statement adopted; 2025 deck retired externally (FACTS_BASE A4)
G4 — Conflict	Any competitor or incumbent	Conflict register entry; partner-first wording approved (§5.5)
G5 — Payment integrity	Any government, official or referral arrangement	Bribery Act 2010 check; written fee terms; no payments to officials

Services and E-Learning partnerships pass G1 and G3 only. Scores are re-run after every first meeting; a partner that fails two consecutive next actions drops one tier.

3. Scored long-list — 25 target partner organisations

All 25 are real, publicly identifiable organisations. **No relationship exists with any of them (VERIFIED absence)**. Scores are desk ESTIMATES (criteria 1–7 as numbered in §2; the last two are inverse). Model codes refer to §4. Pillar codes as in §1.

Rank	Organisation	Class	1 Strat (20)	2 Rev (15)	3 Cred (15)	4 Access (15)	5 Time (15)	6 Cost (10)	7 Risk (10)	Score	Model	Pillar	Gate	Horizon
1	Health Innovation Network (one regional HIN first; e.g., South London or Manchester)	UK innovation body	5	2	5	5	4	5	4	86	Referral, Research	HS, Con	G1	Now
2	Five X More	UK advocacy (Black maternal health)	5	1	5	4	4	4	3	76	Co-development (paid)	HS, Con	G1, G3	Now
3	Africa's Talking	Messaging gateway (Kenya, pan-African)	4	2	2	5	5	5	4	76	Distribution (rail)	HS	G2	Now
4	Society for Family Health Nigeria	Implementing NGO	5	4	4	3	3	3	3	74	Distribution, Funding, Research	HS, Con	G1, G2	6m
5	DigitalHealth.London Accelerator	Accelerator	4	2	4	4	3	4	5	73	Referral	HS, Adv	G1	Now
6	Nigeria Health Watch	Convenor / advocacy	4	1	4	4	4	4	4	71	Referral, Co-development (content)	HS, Adv	G1	Now
7	MSD for Mothers	Corporate programme funder	5	4	5	2	2	3	3	71	Funding	HS	G1, G2, G4	12m
8	Jhpiego	International NGO	5	4	5	2	2	3	3	71	Distribution, Research, Funding	HS, Con	G1, G2	12m
9	Birthrights	UK rights charity	4	1	4	4	4	4	4	71	Co-development (paid)	HS, Con	G1, G3	Now
10	NPEU, University of Oxford	University research unit	5	2	5	2	2	3	4	67	Research	HS	G1, G2	6m
11	NIHR (ARC, i4i, HealthTech Research Centres)	Research funder	4	3	5	2	2	3	4	66	Funding, Research	HS	G1, G2	12m
12	mDoc (Digital Mom Project)	Incumbent (Ekiti, Lagos)	5	2	3	3	3	4	2	65	Co-development / Distribution (complementarity)	HS	G2, G4	Now (conversation)
13	Tommy's	UK research charity	4	1	5	2	3	4	4	65	Co-development (content), Research	HS, EL	G1, G3	6m
14	Royal College of Midwives	Professional body	4	2	5	2	2	4	4	65	Co-development (content), Distribution (E-Learning)	EL, HS	G1	6m
15	UNFPA (Nigeria and Kenya country offices)	Multilateral	4	3	5	2	2	3	3	64	Funding	HS	G1, G2	12m
16	PATH	International NGO (digital health)	4	3	5	2	2	3	3	64	Research, Distribution	HS	G1, G2	12m
17	King's College London	University	4	2	4	3	2	3	4	63	Research, Distribution (E-Learning)	HS, EL	G1	6m
18	Jacaranda Health	Incumbent (Kenya)	4	2	4	3	2	4	3	63	Co-development (complementarity)	HS	G2, G4	6m (conversation)
19	LSHTM MARCH Centre	University research centre	4	2	4	2	2	3	4	60	Research	HS	G1, G2	6m
20	Gates Foundation / Grand Challenges	Foundation	4	4	5	1	1	2	3	59	Funding	HS	G1, G2	12m
21	Wellbeing of Women	UK research charity	3	2	4	2	2	4	4	58	Funding (research), Co-development	HS	G1	12m
22	System C (BadgerNet Maternity)	UK EPR vendor	4	3	3	2	2	3	3	58	Data (integration), Distribution	HS	G2	12m
23	MSI Reproductive Choices	International NGO	3	3	4	2	2	3	3	57	Distribution, Funding	HS, Con	G1, G2	12m
24	FCDO (UK-funded health programmes)	Bilateral funder	3	3	4	2	1	2	3	52	Funding (via primes)	HS, Con	G1	12m–24m+

Rank	Organisation	Class	1 Strat (20)	2 Rev (15)	3 Cred (15)	4 Access (15)	5 Time (15)	6 Cost (10)	7 Risk (10)	Score	Model	Pillar	Gate	Horizon
25	MTN Nigeria / MTN Foundation	Telecom	3	3	3	1	1	3	3	48	Distribution (bundle/white-label)	HS	G2	24m+

Watch list (next ten, not yet scored; re-score at month 6): HCA Healthcare UK (private maternity pilot market); Helium Health (integration in Nigerian private hospitals); Safaricom / M-TIBA (Kenya rails); Wellbeing Foundation Africa (Nigerian maternal-health NGO); Amref Health Africa (Kenya); Best Beginnings (Baby Buddy content signposting); K2 Medical Systems and Magentus (EPR integration); AXA Mansard Health or Hygeia HMO (Nigeria HMO discovery); Bupa or AXA Health (UK benefits discovery); Innovate UK and SBRI Healthcare (funders rather than partners — tracked in [/docs/08_FINANCIAL_MODEL.md](#) grant calendar).

3.1 Reading the ranking

Tier	Organisations	Why	What happens in the next 90 days
Tier 1 — engage now (score ≥ 70)	HIN, Five X More, Africa's Talking, SFH Nigeria, DigitalHealth.London, Nigeria Health Watch, MSD for Mothers, Jhpiego, Birthrights	Reachable, cheap, and each opens a gate	Applications (HIN, DHL); paid co-design proposals (Five X More, Birthrights); gateway account (Africa's Talking); introductions sought (SFH, NHW); MSD for Mothers and Jhpiego prepared but held behind G2
Tier 2 — prepare, engage at 6 months (60–69)	NPEU, NIHR, mDoc, Tommy's, RCM, UNFPA, PATH, King's, Jacaranda, LSHTM	High credibility, slower institutions; most need the entity, a clinical adviser or IP resolution	Desk research; identify the right programme or call; warm-introduction paths mapped; mDoc/Jacaranda conversations as soon as G2 and G4 pass
Tier 3 — track (< 60)	Gates/Grand Challenges, Wellbeing of Women, System C, MSI, FCDO, MTN	Long cycles, high cost, or only relevant after evidence exists	Calendar of calls; no outreach beyond a courtesy contact

The ranking is sensitive to one variable: **if G2 (IP) is resolved by day 30, mDoc, Jhpiego, SFH and MSD for Mothers all move up a tier because time-to-value improves; if it is not resolved by day 90, every HS-pillar partner freezes and the plan collapses to HIN, community co-design and E-Learning partnerships.**

4. Partnership models

Model	Structure	Economics (ESTIMATE)	Default contract form	Best partner types	When to use	Principal risks and controls
Referral	Partner introduces Vytalix to buyers (or vice versa); no delivery obligation	Introduction fee 5–10% of first-year fees, or reciprocal referrals with no fee; HINs and accelerators take no fee	Referral agreement or programme terms; fee disclosed to the customer	HINs, accelerators, consultancies, diaspora associations, convenors	Services growth; NHS introductions	Bribery Act and NHS conflict-of-interest exposure — fees written, disclosed, never to public officials; register entry
Co-development	Joint design, content or research with shared outputs	Vytalix pays community partners (£2k–£10k per engagement, ESTIMATE); joint IP terms written before work starts	Collaboration agreement with IP schedule; content licence	Five X More, Birthrights, MNVPs, Tommy's, RCM, mDoc/Jacaranda (complementarity)	MaternaLink content and safety-critical journeys; C8 listening exercises	IP ambiguity — ownership/licence stated per deliverable; consent and safeguarding for lived-experience work; no "endorsement" claims
Distribution	Partner carries MaternaLink (in development) or E-Learning to its cohort, members or customers	Revenue share 10–30% to the partner or programme licence to the partner (£8k–£60k/programme/yr, /assets/PARTNERSHIP_DECK.md slide 7); telco white-label £25k–£75k set-up + £2–£8/user/yr (24m+)	Reseller/programme agreement; data-processing agreement; SLA	Implementing NGOs, HMOs, telecoms, private hospital groups, RCM (E-Learning), EPR vendors	African cohorts; E-Learning scale; telco channel (24m+)	Dependency on a single channel; data-controller/processor split; local data residency (Nigeria NDPA, Kenya ODPC)
Data	Integration, data exchange or shared datasets	Usually no fee; integration cost borne by Vytalix (v2); no individual-level data to insurers	Data-sharing agreement; DPIA; integration agreement	EPR vendors, Helium Health, state DHIS2 owners, insurers (aggregate only)	v2 integration; state data ownership; adherence dashboards	Regulatory exposure — DPIA before any exchange; state owns programme data; no processing of real patient data in the prototype until audited (FACTS_BASE \$2)
Research	Co-applicant on evaluation or research; shared authorship; pre-registered protocols	Grant-funded; Vytalix costs recovered at 70–100% of eligible costs (scheme-dependent, TO VALIDATE); no fee to the university	Collaboration agreement; ethics (HRA/REC or NHREC) before data; publication policy	NPEU, King's, LSHTM, NIHR ARCs, PATH	UK service evaluation (2027), feasibility study (2028), v3 research track (data partnership only after v1 evidence)	Timeline (12–36 months); authorship disputes; v3 claims discipline (no predictive claims before validation and MHRA pathway)
Funding	Grants, programme funding, corporate/foundation support	£25k–£500k per award (ESTIMATE); success rates single-digit to ~20% (ESTIMATE); match funding 30–50%	Grant agreement; consortium agreement	NIHR, Innovate UK/SBRI, Grand Challenges, UNFPA/UNICEF calls, MSD for Mothers, Wellcome, FCDO via primes	Non-dilutive R&D and evaluation	Eligibility (UK entity; NHS or NGO partner); subsidy control; reporting burden; MSD for Mothers conflict with its existing grantee

Model	Structure	Economics (ESTIMATE)	Default contract form	Best partner types	When to use	Principal risks and controls
Variant — Sub-contracting	Vytalix delivers days inside a prime's programme	Day rate £900–£1,200 (associate rates in /docs/01_BUSINESS_MODEL.md §3)	Sub-contract with IP and non-solicit terms	Consultancies, HINs (evaluation), international NGOs	First NHS/donor references	Margin; being typecast; IP leakage of Vytalix methods
Variant — Content licensing	Vytalix licenses in signed-off content rather than authoring clinical content	Licence fee or free-with-attribution; clinical sign-off by the licensor	Content licence; attribution; update obligations	RCOG patient information, Tommy's, Best Beginnings, WHO ANC digital adaptation kit (open)	MaternaLink education layer; E-Learning	Version drift; licensor's clinical-governance requirements; no re-writing clinical content without sign-off

Model selection rule (PROPOSED): for each partner, choose the model with the shortest path to a signed document, then add models over time. A community organisation starts as paid co-development and may become a distribution partner; an NGO starts as a joint-bid research/funding partner and becomes a distribution partner when a cohort exists; a HIN starts as referral and becomes an evaluation (research) partner.

5. Twelve-month partnership plan (PROPOSED)

Month 1 = the month of incorporation (planned September–October 2026 per [/docs/18_90_DAY_EXECUTION_PLAN.md](#)). Everything below depends on gates G1–G5 in §2.

5.1 Quarter plan and targets

Quarter	Focus	Partner actions	Documents targeted by quarter-end	Founder days (ESTIMATE)
Q1 (months 1–3)	Access and co-design; gate-clearing	Apply to one regional HIN programme and DigitalHealth.London; paid co-design proposals to Five X More and Birthrights; Africa's Talking developer account and pricing; introduction requests to SFH Nigeria and Nigeria Health Watch via public routes; Ekiti status call; mDoc complementarity email only if G2 passes ; advisory-board recruitment via diaspora associations	1 programme application submitted; 1 paid co-design agreement signed; gateway account live; 2 advisory-board letters (per /docs/18_90_DAY_EXECUTION_PLAN.md 3.11); partnership register created	12
Q2 (months 4–6)	Evidence and Nigeria route	University conversations (NPEU, King's, LSHTM) about a 2027 service evaluation; NIHR ARC engagement; second co-design agreement; RCM content/CPD conversation; SFH Nigeria or Jhpiego LOI for a cohort scoping; Jacaranda conversation; state scorecard field scan (Oyo, Kano) with in-country partners; Tommy's content conversation	1 university letter of intent to co-apply; 1 NGO LOI to scope a cohort; 2 paid co-design agreements in total; mDoc and Jacaranda conversations documented	15
Q3 (months 7–9)	Funding and design partner	First two grant bids (e.g., SBRI Healthcare or Innovate UK; a Grand Challenges or UNFPA call — calendars TO VALIDATE) with a university or NGO lead; NHS design-partner conversation opened through a clinical adviser and the HIN; MSD for Mothers approach with complementarity framing (post-G2, G4); private maternity pilot conversation (HCA UK-type unit)	2 bids submitted; 1 NHS trust design-partner conversation at "exploring DCB0160 with CSO"; 1 private-provider pilot LOI	18
Q4 (months 10–12)	Convert and institutionalise	Convert one design-partner conversation to a written evaluation plan; convert NGO LOI to a programme agreement or joint bid; E-Learning distribution conversation with RCM or a university programme; telco/HMO discovery interviews (5); re-score the long-list; partnership board paper	1 design-partner agreement or evaluation plan; 1 programme agreement or funded bid; 1 E-Learning distribution LOI; long-list re-scored	15

Twelve-month scorecard (PROPOSED): 1 HIN/accelerator programme place · 2 paid community co-design agreements · 1 university co-applicant agreement · 1 Nigerian implementing-partner LOI or agreement · 1 gateway pilot account · 1 NHS design-partner conversation opened (agreement is a stretch) · 2 joint grant bids submitted · 1 E-Learning distribution LOI · 0 unverified "partner" claims in any material (audited monthly).

5.2 Outreach sequence (per partner; adapt to model)

Step	Timing	Action	Rule
1	Day 0	Qualify: score in §2; check gates; identify the decision-maker title and a public route (programme application, published contact, event, LinkedIn); log in CRM with trigger URL	No invented contact details
2	Day 0–3	Earn the right: read the organisation's latest report, strategy or programme page; identify one thing Vytalix can offer that costs them nothing (a data point, a draft, an introduction, a paid brief)	Never open with an ask for endorsement or a logo
3	Day 3	First message (≤ 120 words): who Vytalix is (pre-seed, honest stage), why this organisation specifically, the one offer, one question, a 20-minute ask	Use the "what we offer / what we ask" line for the partner type (§5.3); language rules from FACTS_BASE §4
4	Day 10	Follow-up 1: add one piece of value (a relevant excerpt from the sourced report in /docs/14_MARKETING_AND_CONTENT_ENGINE.md , a public call they may not have seen)	One follow-up; no pressure
5	Day 20	Follow-up 2 or alternative route: an event, a mutual contact found through public sources, or a programme application instead of a cold message	Three touches maximum, then park for 90 days
6	Meeting	Discovery, not pitch: 60% listening; confirm what they control, their calendar (calls, cohorts, board dates), their conditions; agree one concrete next step with a date	Bring the artefact relevant to the model (co-design brief, integration note, evaluation outline, bid outline)

Step	Timing	Action	Rule
7	+7 days	Written follow-up: one-page note of what was agreed; proposed document type (LOI/MoU/agreement/application); named next action on both sides	Nothing is a partnership until signed; wording of any public reference agreed in writing
8	Monthly	Register review: status, next action, gate status, any public reference permission	Founder + (later) a partnerships lead

5.3 What we offer / what we ask — by partner type

Partner type	What we offer	What we ask	What we will not do
NHS trusts / LMNS	Vendor-neutral readiness and inequalities reviews (C3, C8); a non-diagnostic communication and handover layer beside the EPR with the safety artefacts (DCB0129 hazard log, DPIA, intended-use statement) supplied; a service-evaluation design; no licence fee for the design partner	A digital midwife and a Clinical Safety Officer willing to run DCB0160; one community team; 3–6 months; a service-evaluation write-up	Replace the EPR; make clinical claims; use the trust's name without written permission
Health Innovation Networks / accelerators	A maternal-health innovation with an honest evidence plan and a founder who will do the work; participation in their evaluation and reporting; case-study material once evidence exists	A programme place; introductions to two or three maternity digital leads; evaluation methodology support; no fee	Present the programme as an endorsement; ask them to sell for us
Universities / NIHR	A real-world deployment to study; co-designed protocols; shared authorship; SME co-applicant status; industry match where schemes require it	Methodological partnership; a named principal investigator; co-application on one call in 2027; ethics route guidance	Train models on patient data without approvals; make predictive claims; treat the v3 research track as a product commitment
Charities and community organisations	Payment for co-design, listening exercises and content review; credit in outputs; influence over the woman-facing journeys; joint bids where they lead	Lived-experience input; content review; participation in a C8 listening exercise; candid criticism	Ask for endorsement, logos, free advocacy or unpaid time; speak on their behalf
Professional bodies (RCM, RCOG)	CPD-ready E-Learning aligned to their standards (accreditation TO VALIDATE); content licensing revenue where relevant; speaking content on digital maternity	A content-licensing conversation; guidance on CPD routes; conference slots	Claim accreditation or endorsement before it exists
Implementing NGOs (Nigeria, Kenya)	Enrolment and follow-up tracking, two-way SMS at scale, CHEW defaulter lists, logframe/DHIS2-compatible exports, data owned by the NGO or state, local-hosting option (TO VALIDATE), a programme licence priced to a grant line (£8k–£60k/yr, ESTIMATE), joint bid capacity from the UK	An existing cohort in 1–2 LGAs; CHEW supervision; a budget line or joint grant; co-authorship of the evaluation; NHREC/state ethics	Arrive with a state-wide proposal; compete on price with an incumbent programme; share individual data with third parties
Donors, foundations and multilaterals (MSD for Mothers, Gates/Grand Challenges, UNFPA, UNICEF, FCDO primes)	A complementary, partner-first coordination layer for existing programmes; transparent total programme cost; evaluation-grade data; UK–Nigeria/Kenya corridor delivery with a UK entity	Programme-officer conversation; call eligibility guidance; consideration as a sub-grantee or consortium member; introductions to grantees	Pitch against an existing grantee (mDoc); restate the Ekiti proposal; claim any relationship
African governments (via partners)	An LGA-level pilot under £0.5m/yr (ESTIMATE) with a 12-month evaluation and defined exit; data ownership with DHIS2 export; NDPA compliance; a local implementing partner	An LOI, then an MoU, then a pilot agreement — in that order; a named focal person; access to CHEWs and facilities	Approach alone; describe an MoU as a government partnership; make payments to officials or intermediaries
Incumbents (mDoc, Jacaranda, Helium Health)	Complementary facility-side coordination, referral tracking and handover; integration-ready posture; shared learning; a joint offer to donors where it helps them	One exploratory conversation; clarity on where they see gaps; an integration or referral conversation if there is fit	Enter their state/county against them on price; imply a partnership after one call
Telecoms and gateways	A maternal-health messaging and coordination use case with programme buyers; volume assumptions (~60 SMS/woman/year, ESTIMATE); a white-label option later	Gateway pricing and sender-ID/DND guidance (NCC rules TO VALIDATE); zero-rating or bundled data for enrolled women; a named pilot contact	Pass identifiable health data; commit volumes before a funded pilot
Insurers and HMOs	Aggregate adherence dashboards; engagement-fee pricing (24m+); reduced no-shows for covered women (to be evidenced)	Discovery interviews now; a benefits-bundle conversation after evidence	Share individual clinical data; perform underwriting or risk scoring
Private hospital groups	A fast, low-IG-barrier pilot with the safety artefacts included (£10k minimum bundle per /docs/13_SALES_ENGINE .md); a C1 discovery sprint; staff E-Learning	A lead obstetrician sponsor; one service line; a paid pilot	Undercut on price; skip DPIA and safety case because the setting is private
Consultancies (primes)	Specialist maternity/digital/Africa depth at associate rates; speed; fixed-price sub-scopes	Sub-contract days on NHS or donor programmes; a framework route	Compete for their prime work; use their client names

5.4 Ekiti and mDoc — the specific plan

Step	Timing	Action	Owner
1	Q1, by day 26	One call to learn the status of the March 2026 proposal (live, dead, in negotiation — FACTS_BASE \$2). No restatement of the proposal. Outcome recorded in FACTS_BASE.	Founder (with David Agunede, subject to the IP conversation)
2	Q1, only after G2 and G4	Complementarity email to mDoc (wording rules in \$5.5). One exploratory conversation.	Founder
3	Q1–Q2	Two-state scorecard field scan (Oyo, Kano) with in-country NGO conversations (/docs/05_MARKET_STRATEGY .md \$2.1).	Founder + in-country associate (TO RECRUIT)
4	Day 60	Go/no-go: partner / differentiate / move state (/docs/18_90_DAY_EXECUTION_PLAN .md G-D).	Founder + advisory board
5	Q3	MSD for Mothers approached only with a complementarity framing agreed (or not objected to) by mDoc, or with a different-state, NGO-led proposal.	Founder

5.5 Wording rules for incumbent and donor outreach (mDoc, Jacaranda, MSD for Mothers)

- State the stage honestly: "pre-seed, UK-based, MaternaLink in development, no deployments".
- Acknowledge their programme explicitly and by its public name; cite the public source of what we know.
- Position on **what they do not do** (facility-side coordination, referral tracking, handover, CHEW defaulter lists, evaluation-grade exports) — never on price, never on "better".
- Ask one question: "Where do you see gaps a complementary coordination layer could fill, and would a 30-minute conversation be useful?"
- Never mention the Ekiti proposal, its pricing, or any state contact; never imply a state relationship.
- No public reference to the conversation afterwards; the CRM entry is the only record.
- If they decline or do not reply, the fallback is "move state" or "different layer", not repetition.

5.6 Governance

Instrument	Content	Owner	Cadence
Partnership register	Organisation, class, model, gate status, document status (target/conversation/LOI/MoU/agreement), fee terms, public-reference permission, conflicts, next action	Founder	Updated within 24h of contact; reviewed monthly
Document ladder	LOI (non-binding, 1 page) → MoU (non-binding, scope and principles) → agreement (binding: collaboration, referral, reseller, data-sharing, sub-contract, grant/consortium)	Founder; solicitor for binding documents	Per partner
Conflicts register	Competitors advised, incumbents contacted, referral fees, public officials met	Founder	Monthly
Public-reference protocol	No name, logo or "in discussion" without written permission; wording approved by the partner	Founder	Per reference
Anti-bribery and gifts policy	Bribery Act 2010; no facilitation payments; hospitality thresholds; agent due diligence in Nigeria/Kenya	Founder	Adopted at incorporation; reviewed annually
Partner review	Score re-run, pipeline by tier, documents signed, days spent, lessons	Founder + advisory board	Quarterly

5.7 Budget (ESTIMATE, 12 months)

Item	Amount	Note
Paid community co-design (2 agreements)	£6k–£16k	£3k–£8k each; the single most important spend in this plan
Programme and application fees	£0–£2k	Most HIN/accelerator routes are free (TO VALIDATE)
Messaging gateway pilot credits	£500–£2k	Africa's Talking-type account; pass-through in pilots
In-country field scan (two states)	£4k–£10k	Associate days, travel; may be partly covered by a C5-type paid engagement
Legal (LOI/MoU templates, collaboration and referral agreements)	£3k–£8k	Templates once; reuse
Travel and events (UK conferences, one Nigeria trip, one Kenya desk phase)	£4k–£8k	Nigeria travel only after G2 and an NGO conversation
Founder time	~60 days	~25% of available days; in tension with ~87 billable days (/docs/01_BUSINESS_MODEL .md \$4.2)
Total cash	£18k–£46k	Funded from services revenue and, from Q3, grant cost recovery

Priorities / Risks / Next actions

Priorities

1. Clear the gates before the outreach: entity (G1), IP with David Agunede (G2) and the claims re-base (G3) are partnership prerequisites, not parallel tracks. Services and community co-design proceed under G1/G3 only.
2. Engage Tier 1 in the first 90 days: one HIN programme application, DigitalHealth.London application, two paid co-design proposals (Five X More, Birthrights), a gateway account, and public-route introductions to SFH Nigeria and Nigeria Health Watch.
3. Treat mDoc and Jacaranda as partner tests: one documented complementarity conversation each before any further product spend in Nigeria or Kenya.
4. Build the evidence coalition early: a university co-applicant and a HIN evaluation route by month 6, so that 2027 grant bids have partners named.
5. Keep every partner claim auditable: partnership register, public-reference protocol and a monthly zero-unverified-claims check across website, decks and investor material.

Risks

- **IP unresolved** → every Health Solutions partnership freezes; mitigation: the plan degrades gracefully to HIN, co-design and E-Learning partnerships, and the day-30 and day-90 decision points are explicit.
- **Incumbent reads outreach as competitive** (mDoc, MSD for Mothers) and closes Nigeria doors; mitigation: \$5.5 wording rules, no Ekiti restatement, "move state" fallback with Oyo/Kano scored.
- **Founder capacity**: ~60 partnership days compete with ~87 billable days and fundraising; mitigation: Tier 1 only in Q1; an in-country associate and a fractional partnerships lead from Q3 if services revenue allows.

- **Community organisations approached as free endorsement** by a start-up with no track record damages reputation permanently; mitigation: paid co-design only; written credit; no logos.
- **Grant dependence:** two bids at single-digit to ~20% success rates (ESTIMATE) may yield nothing; mitigation: bids are a by-product of partnerships, not the plan's spine; services revenue funds the year.
- **Regulatory or data exposure through a partner** (e.g., a gateway or NGO handling data without a DPA, or the un-audited prototype used in a demo with real data); mitigation: DPIA and data-sharing agreements before any data; prototype audit (/docs/04_MATERNALINK_PRODUCT_STRATEGY.md \$25) before any partner demo.
- **Bribery Act and NHS conflict-of-interest exposure** from referral fees or in-country intermediaries; mitigation: \$0 rule 5, \$5.6 policy, no payments to officials, agent due diligence.

Next actions

- Day 1–7: create the partnership register and conflicts register; write the one-page LOI and MoU templates; identify the regional HIN and its current programme calendar; draft the two paid co-design briefs (£3k–£8k each).
- Day 7–14: submit or diarise the HIN and DigitalHealth.London applications (once incorporated); open the Africa's Talking-type gateway account; send introduction requests to SFH Nigeria and Nigeria Health Watch via public routes; hold the Ekiti status call (by day 26 at the latest).
- Day 14–30: send the co-design proposals to Five X More and Birthrights; map university contacts (NPEU, King's, LSHTM) and NIHR ARC programmes; recruit the first two advisory-board members through diaspora and clinical networks.
- Day 30: G2 decision point — if the IP agreement is signed, send the mDoc complementarity email and open Jhpiego/SFH cohort conversations; if not, hold all Health Solutions outreach and record the decision.
- Day 60: Ekiti go/no-go with the two-state scorecard; first re-score of the long-list; update /docs/06_CUSTOMER_SEGMENTS.md \$3.4 on the design-partner route (NHS trust vs private maternity vs NGO).
- Day 90: quarterly partner review; Q2 plan confirmed; partnership section of the investor narrative updated with documents only — never conversations.